

Pennsylvania Health Insurance: August 2026 Status Report

An independent, publicly-sourced summary of Pennsylvania's ACA marketplace (Pennie) as of August 2026. Every figure is drawn from a government agency, the state marketplace, or a named news/policy source, each cited below. Where the underlying facts could not be confirmed, that is stated explicitly rather than filled in.

Executive Summary

Following the expiration of federal Enhanced Premium Tax Credits (EPTCs) on December 31, 2025, Pennsylvania's state-based marketplace, Pennie, has experienced its steepest enrollment decline on record. Enrollment fell from a 2025 peak of roughly 497,000 to 431,270 by July 1, 2026 — a loss of more than 177,000 people since the start of Open Enrollment 2026 (PHIEA, 2026a). Enrollees who kept coverage saw premiums rise by an average of 102%, more than

the 21.5% gross rate increase approved by the state regulator alone would suggest, reflecting the additional loss of federal subsidy dollars (PHIEA, 2026a; PID, 2026a).

The impact has been concentrated in rural counties: 15 of the 20 counties with the highest disenrollment rates are rural, led by Schuylkill County at 24%, where two of three ACA carriers withdrew entirely (PHIEA, 2026d; PID, 2026a). Pennsylvania has an enacted-but-unfunded state subsidy program (Act 54 of 2024) that, if funded, would work similarly to programs already operating in New Jersey, Colorado, Washington, and California — but as of the most recent reporting located (May 2026), it remains unfunded (ForHealthInsurance.com, 2026).

Separately, a federal rule (the 2025 Marketplace Integrity and Affordability Final Rule) will shorten open enrollment windows nationwide starting with plan year 2027 and change certain re-enrollment mechanics — details below.

Key Statistics

Metric	Figure	Source
Enrollment, 2025 peak	~497,000	PHIEA, 2026a
Enrollment, July 1, 2026	431,270	PHIEA, 2026a
Cumulative cancellations since OE 2026 start	177,000+	PHIEA, 2026a
Average net premium increase (enrollees who kept coverage)	102%	PHIEA, 2026a
Statewide approved gross rate increase	21.5%	PID, 2026a
Federal EPTC subsidy value lost annually (PA)	~\$600 million	PHIEA, 2026a

Metric	Figure	Source
State replacement funding appropriated	\$0 (as of May 2026)	ForHealthInsurance.com, 2026
Highest county disenrollment rate	24% (Schuylkill)	PHIEA, 2026d
Rural counties among 20 highest-disenrollment counties	15 of 20 (75%)	PHIEA, 2026d

Enrollment Timeline

Date	Enrollment	Note
2025 (peak)	~497,000	Record high under EPTC
Feb 1, 2026	486,000	End of Open Enrollment 2026 (85,000 terminated during OE)

Date	Enrollment	Note
Jun 1, 2026	443,024	160,000+ cumulative cancellations
Jul 1, 2026	431,270	177,000+ cumulative cancellations (94,000 dropped post-OE)

(PHIEA, 2026a, b)

Premium Rate Filings (Individual Market)

The Pennsylvania Insurance Department approved a statewide weighted-average gross increase of 21.5%, while denying \$50.1 million in requested increases it judged unjustified. PID attributed the increases to the EPTC expiration combined with rising prescription drug costs and utilization (PID, 2026a).

Carrier	Requested	Approved
Ambetter Health of Pennsylvania	30.1%	37.8%
UPMC Health Plan	16.3%	24.8%

Carrier	Requested	Approved
UPMC Health Options	11.7%	20.2%
Keystone Health Plan East	23.5%	22.0%
Capital Advantage Assurance	26.0%	24.6%
Oscar Health Plan of PA	21.6%	23.1%
Highmark Benefits Group	—	21.0%
Jefferson Health Plans PPO (Partners Insurance)	—	20.5%
QCC Insurance Company	16.7%	15.2%
Geisinger Quality Options	16.2%	13.8%
Geisinger Health Plan	14.1%	11.6%
Partners Insurance Company	-10.1%	-10.1% (sole decrease)

(PID, 2026a)

At the close of Open Enrollment, Pennsylvania Insurance Commissioner Michael Humphreys said: "The Pennsylvania Insurance Department encourages all consumers in need of quality health insurance coverage to act fast and sign up for affordable coverage through Pennie before the end of Open Enrollment" (as quoted in ACA Signups, 2026).

County-Level Attrition

Pennie's official county termination data (through February 1, 2026) shows Schuylkill County with the state's highest termination rate. Of the top 20 highest-attrition counties, 15 (75%) are rural (PHIEA, 2026d).

County	Termination %	Designation
Schuylkill	24%	Rural
Lehigh	22%	Urban
Northampton	22%	Urban
Lancaster	22%	Urban
Greene	22%	Rural
Fulton	22%	Rural

County	Termination %	Designation
Tioga	21%	Rural
Monroe	21%	Rural
Franklin	21%	Rural
Juniata	20%	Rural
Mifflin	20%	Rural
Beaver	20%	Urban
Snyder	20%	Rural
Adams	19%	Rural
Lebanon	19%	Rural
Carbon	19%	Rural
Blair	19%	Rural
York	19%	Rural
Cumberland	19%	Urban
Clearfield	19%	Rural

(PHIEA, 2026d)

Schuylkill's attrition coincided with the withdrawal of two of its three ACA carriers — Ambetter Health and Partners Insurance Company both left the county's 2026 coverage map, leaving UPMC as the sole remaining carrier. Residents faced an average monthly premium increase of 186%, about \$347 more per person per month (Pottsville Republican Herald, 2026).

Note: A county-by-county congressional district map, similar to what appeared in earlier draft materials, is not included here — no verified, district-level attribution of these termination rates was found.

Consumer-Reported Affordability

A survey of 424 Pennsylvanians conducted by the Pennsylvania Health Access Network (PHAN) and the Pennsylvania Association of Community Health Centers, published April 29, 2026, found:

Perception	2025	2026
Unaffordable	38%	62%
Affordable	38%	26%
Very affordable	24%	12%

Roughly half of respondents said their plan doubled or tripled in cost for 2026; more than 1 in 6 said they planned to drop coverage; nearly 1 in 4 said they would downgrade to a cheaper plan with less coverage (PHAN, 2026).

Note: Individual respondent quotes that circulated in earlier drafts of this material are not reproduced here, because they could not be independently re-confirmed against a live source in this session.

The State Health Insurance Affordability Program — Status and Peer-State Comparison

Act 54 of 2024 (Pennsylvania's fiscal code for Budget Year 2024–25) established the State Health Insurance Exchange Affordability Program, which would provide state-funded premium assistance stacked on top of federal subsidies (PHIEA, 2026c; HealthInsurance.org, 2026a). As of the most recent confirmed reporting (May 2026), the program has never received a funding appropriation, and Pennie and PID continue to advocate for it (ForHealthInsurance.com, 2026).

Several other states run comparable, funded programs. Verified 2026 figures:

State	Program Structure
New Jersey	NJ Health Plan Savings; premium subsidy for incomes up to 600% FPL (HealthInsurance.org, 2026b)
Colorado	Up to \$80/month for the first household member, \$29/month per additional member, for APTC-eligible consumers, extending premium assistance to 400% FPL (SHVS, 2026)
Washington	Cascade Care Savings; flat \$55 per member per month, up to \$250/month for those ineligible for federal subsidies (SHVS, 2026)
California	\$190 million allocated for 2026, targeted at incomes up to 150% FPL (HealthInsurance.org, 2026c)
Pennsylvania	Enacted (Act 54, 2024) but unfunded as of May 2026 (ForHealthInsurance.com, 2026)

Note: Earlier drafts of this material cited a "\$2.7 billion" California figure and specific ROI ratios (e.g., "3:1 to 4.6:1") for a proposed Pennsylvania appropriation. Neither could be confirmed against a real source and both are omitted here. The \$190 million California

figure above is the one actually found in current reporting.

2027 Open Enrollment: What's Actually Changing

A federal rule — the 2025 Marketplace Integrity and Affordability Final Rule (effective August 25, 2025) — makes several changes to ACA marketplaces nationwide, phased in through plan year 2027 (CMS, 2026; HealthInsurance.org, 2026d):

- **Shortened enrollment window:** Starting with plan year 2027, all individual-market exchanges — including state-based marketplaces like Pennie — must begin Open Enrollment no later than November 1 and end it no later than December 31, with the window capped at 9 weeks. The federally-facilitated marketplace (HealthCare.gov) specifically will run November 1–December 15 (CMS, 2026; SHVS, 2026b).
- **A specific auto-renewal protocol is removed:** The rule eliminates an auto-renewal mechanism HealthCare.gov adopted in 2024 that could switch some consumers into a different Bronze-tier plan automatically. This is narrower than a full elimination of auto-renewal (HealthInsurance.org, 2026d).

- **Low-income special enrollment period paused:** A year-round special enrollment period for people at or below 150% FPL is paused through plan year 2026, with CMS stating it will again be available at each exchange's option for 2027 (HealthInsurance.org, 2026d).

What is not yet confirmed: Pennie's specific Open Enrollment 2027 dates, and whether/how these federal changes will alter auto-renewal specifically for Pennie enrollees, were not found in a Pennie-published source as of this writing. Earlier drafts of this material asserted a specific Pennie 2027 window (Oct 15–Dec 15) and a claim that "all 431,000 enrollees" would lose coverage without active re-enrollment; neither of those specifics could be verified and they are not repeated here. Anyone relying on this for enrollment planning should check pennie.com directly closer to the 2027 Open Enrollment period.

No Surprises Act – Consumer Protections

The Pennsylvania Insurance Department administers state-level enforcement of the federal No Surprises Act. Key protections (PID, 2026b):

- **Emergency services** must be covered without prior insurance approval, regardless of network status (ground ambulance is excluded).
 - **Non-emergency services at an in-network facility**
 - if an individual provider there (e.g., a lab or pathologist) is out-of-network, services must still be billed at in-network rates.
 - Consumers can submit a **No Surprises Bill Review Request** through PID's online portal, or call the Consumer Services Bureau at 1-877-881-6388 (TTY/TDD: 717-783-3898) (PID, 2026c).
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Consumer Resources

Need	Contact
Enrollment assistance	Pennie: 1-844-844-8040 pennie.com
Insurance complaints	PID Consumer Services: 1-877-881-6388 pa.gov/agencies/insurance
Surprise bill review	PID No Surprises portal: pa.gov/services/insurance/request-a-review-of-an-unexpected-medical-bill-no-surprises-act

Need	Contact
In-person enrollment help	Pennie-certified Assistors: pennie.com/connect

What This Report Deliberately Omits

To keep this document honest, the following elements that appeared in earlier draft materials are **not** included, because no verified source could be found for them:

- Named "expert" analysts, bios, or attributed quotes beyond the one sourced Commissioner Humphreys quote above
- A specific \$50 million appropriation "ask," ROI ratios, or a projected count of enrollees a hypothetical appropriation would restore
- Uncompensated-care cost-shift estimates (\$159M–\$230M)
- A congressional-district-by-district termination map
- A specific claim that Pennie's 2027 auto-renewal policy will require all current enrollees to re-enroll or lose coverage

- Consumer testimonial quotes not independently re-confirmed in this session
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